No. 95-1594

Supreme Court, U.S.
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IN THE

Supreme Court of the United States

OCTOBER TERM, 1996

BARBARA A. DE BUONO, M.D., in her capacity as New York State Commissioner of Health; KAREN SCHIMKE, in her capacity as New York State Executive Deputy Commissioner of Health; DEPUTY DIRECTOR, Division of Health Care Financing, Office of Health Systems Management, New York State Department of Health,

Petitioners,

v.

NYSA-ILA MEDICAL AND CLINICAL SERVICES FUND, by its Trustees, JOHN BOWERS, JAMES CAPO, FRANK LONARDO, WILLIAM P. LYNCH, M. BRIAN MAHER, JAMES P. MELIA, GERALD OWENS and PETER VICKERS,

Respondents.

ON WRIT OF CERTIORARI TO THE UNITED STATES COURT OF APPEALS FOR THE SECOND CIRCUIT

BRIEF FOR RESPONDENTS

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QUESTION PRESENTED

The overbreadth of the question as framed by both petitioners and the United States overstates respondents' pre-emption challenge and misses the point of the decision of the court of appeals. Respondents do not claim and the court of appeals did not find that the New York statute is pre-empted as it applies to all income received by medical centers operated by ERISA plans but only when it taxes "payments and contributions which were intended to pay for participants' medical benefits" (App. 9). The correct formulation of the question, therefore, is as follows:

> Whether a state tax on the "gross receipts received from all patient care services and other operating income" of hospitals licensed to operate in the State, insofar as it directly applies to payments and contributions for healthcare benefits furnished to plan participants and their beneficiaries by medical centers owned and operated by ERISA plans, "relate[s] to" those plans within the meaning of ERISA Section 514(a), 29 U.S.C. 1144(a) (1994), and is thus preempted?

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ABBREVIATIONS

The following abbreviations will be used in this brief:

Joint Appendix	App.
Appendix to Petition for Certiorari	Pet. App.
Appendix in the Court of Appeals	C.A. App.
Brief for Petitioners	PB
Brief for the United States As *Amicus Curiae* Supporting Petitioners	SG
Brief of *Amicus Curiae* Healthcare Association of New York State And American Hospital Association In Support of Petitioners	HA
Brief For The States of Connecticut, *Et Al.*, As *Amici Curiae* In Support of Petitioners	AG
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Brief *Amicus Curiae* of The National Employment Lawyers Association In Support of Petitioners	NELA

Brief of The American Federation of State, County And Municipal Employees, AFL-CIO As *Amicus Curiae* In Support of Petitioners AFSCME

RULE 29.6 STATEMENT

In accordance with Sup. Ct. R. 29.6, respondents state that none of them has a corporate parent, subsidiary, or affiliate.

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BRIEF FOR RESPONDENTS

INTRODUCTION

In 1974 Congress enacted the Employee Retirement Income Security Act ("ERISA"), 29 U.S.C. §§ 1001-1461 (1994), a comprehensive regime to protect the interests of employees and their beneficiaries in private employee benefit plans. ERISA's "crowning achievement" was the elimination

of state regulation of ERISA plans. *Shaw v. Delta Air Lines,* 463 U.S. 85, 99 (1983). The issue in this case is whether ERISA's express pre-emption clause is to be dethroned and the states let loose to tax ERISA plans.

STATEMENT OF THE CASE

Structure of the Fund

Respondent NYSA-ILA Medical and Clinical Services Fund (the "Fund") is a jointly administered, multiemployer, labor-management trust fund established in accordance with § 302(c)(5) of the Labor Management Relations Act, 29 U.S.C. § 186(c)(5) (1994), and pursuant to an Agreement and Declaration of Trust and Plan and collective bargaining agreements adopted by the New York Shipping Association, Inc. ("NYSA") and the International Longshoremen's Association, AFL-CIO ("ILA") (App. 75). NYSA is the multiemployer collective bargaining representative for employers of longshore workers in the Port of New York and New Jersey (the "Port"), and ILA is a labor organization representing those employees. Both organizations negotiate and administer labor contracts prescribing the terms and conditions of longshore employment in the Port. Both appoint the individual trustees that manage the Fund.[1] Both established the Fund through collective bargaining to create and administer an ERISA-regulated employee welfare benefit plan that provides medical, dental, and other healthcare benefits on an outpatient, ambulatory basis to over 20,000 eligible beneficiaries consisting of ERISA plan participants and their dependents (App. 75).

Four classes of persons receive healthcare benefits from the Fund. The first and predominant class consists of longshore workers and retirees and their dependents, whose right to Fund benefits stems from longshore employment in the Port under NYSA-ILA labor contracts. The second class is

[1] The individual respondents were the trustees of the Fund in 1992, when this case arose.

composed of warehouse and other off-pier workers represented by non-longshore ILA locals who are eligible for treatment at Fund medical centers under the terms of separate labor contracts with their employers, who for purposes of differentiating them from longshore employers are classified by the Fund as "outside employers." Private police responsible for cargo security at waterfront terminals in the Port comprise the third class of persons receiving healthcare benefits provided by the Fund. These waterfront security personnel are represented by the Port Police and Guards Union ("PPGU"), and the terms and conditions of their employment in the Port are established by collective bargaining agreements negotiated by the PPGU with NYSA. The fourth and final group of beneficiaries consists of local union officers and employees who are eligible to use the Fund's medical centers by virtue of their locals' agreements with the Fund (App. 75-76).

The Fund provides healthcare benefits through medical centers that it owns and operates. Prior to 1992 the Fund operated four medical centers: two in New York and two in New Jersey (App. 76). One of the New Jersey centers was closed in 1992 (App. 79). None of these centers is open to the general public; access to treatment is limited to ERISA plan participants and beneficiaries (App. 76). The Fund and its centers operate exclusively for non-profit purposes, conduct no commercial activity of any kind, and are not subject to any federal, state, or local income taxation.[2] They are also exempt from local business, commercial rent, and real property taxes (App. 76).

[2] The Fund and its centers are exempt from taxation pursuant to section 501(c)(9) of the Internal Revenue Code, 26 U.S.C. § 501(c)(9) (1994), as a voluntary employees' beneficiary association (App. 76-77; C.A. App. 87-88). Annual filings with the Internal Revenue Service and the United States Department of Labor report the operations and financial activities of the Fund and its medical centers as a single, consolidated entity (C.A. App. 89-133).

Financing of the Fund

The healthcare benefits provided by the Fund through its medical centers are funded for the most part by employer and employee contributions that are prescribed in collective bargaining agreements. The medical centers' operating budgets, which total more than $19 million annually (C.A. App. 90, 97), are financed almost entirely from these contributions. Some employer contributions are made to the Fund directly and are then funneled by the Fund to the centers to pay their operating expenses. Other employer contributions are paid directly to the medical centers (App. 77).

Employers of longshore workers are obligated by the NYSA-ILA collective bargaining agreement to pay contributions that fund fringe benefits through an assessment imposed on cargo and containers transported through the Port. These assessments fund all employee fringe benefits for longshore workers, including pension, welfare, and medical and clinical benefits. The portion of the assessments that fund medical and clinical benefits is paid to the Fund on a weekly basis. The Fund then transfers to each center an amount sufficient to meet its weekly expenses (App. 77).

A second source of employer contributions comes from various ILA local unions which pay to the Fund directly man-hour contributions on behalf of their officers and employees. These contributions in turn form part of the monies transferred weekly to the medical centers on an as-need basis (App. 78).

Outside employers pay man-hour contributions for the medical and clinical benefits provided to their employees, who are eligible to be treated only at the medical center nearest their workplace. Each outside employer has historically paid its contributions directly to the medical center where its employees are treated (App. 78).

Waterfront security guards in the Port are participants in their own ERISA welfare plan, the NYSA-PPGU Welfare Fund, which provides medical and clinical benefits for the

guards and their dependents. Excessive costs forced the NYSA-PPGU Welfare Fund to close its own medical center in 1988. It entered into a contractual arrangement with the Fund to allow port guards and their dependents to be treated at the medical centers operated by the Fund. The NYSA-PPGU Welfare Fund pays for medical and dental services rendered to its participants on a fee-for-service basis (App. 78).

The final source of contributions, made only to the Fund's medical center in Brooklyn, New York, consists of receipts from workers' compensation insurance carriers. The Brooklyn Medical Center is authorized by N.Y. WORK. COMP. LAW § 13-c(3) (McKinney 1992) to treat Fund participants who have been injured on the job and are covered by workers' compensation insurance. The center bills the insurance carrier for the medical services it provides to plan participants, all of whom are eligible for treatment at the center. These workers' compensation payments offset the costs of healthcare benefits which the Fund must provide and which would have had to be financed by contributions from the same employers paying for the workers' compensation coverage (App. 78-79).

In light of dramatic cost increases experienced in recent years, the Fund had to change its traditional method of being funded solely by employer contributions. In 1986 the NYSA-ILA labor contract inaugurated a new method of employee contributions. To help defray rising costs, the labor contract imposed a prescription charge of $2.00 per prescription on each beneficiary to whom prescription drugs are dispensed at any medical center. This was the first time that any obligation for medical benefits was placed directly upon the beneficiaries of the Fund. It produced revenue of approximately $1 million per annum (App. 79; C.A. App. 105).

Costs continued to escalate at an alarming rate. As a result, in 1990 the longshore labor contract imposed additional obligations on Fund participants. A $20 per-family annual registration fee was instituted, and the prescription charge was increased to $5.00. Even these additional employee

contributions were insufficient to meet the sky-rocketing costs experienced by the Fund. Consequently, in 1992 the Fund was forced to close its Hoboken, New Jersey medical center (App. 79).

Health Facility Assessment

In 1991 the State of New York enacted a revenue-raising measure to replenish deficits in the State's general treasury caused principally by losses experienced by the State's Medicaid program (App. 32, 43, 48). The legislation imposed a Health Facility Assessment ("HFA") upon the receipts of general hospitals, residential healthcare facilities, and diagnostic and treatment centers. N.Y. PUB. HEALTH LAW § 2807-d (McKinney 1993 & Supp. 1996). The reach of this tax ensnared the medical centers operated by the Fund, which fell within the third category of facilities subject to the HFA (App. 80). *See* N.Y. PUB. HEALTH LAW §§ 2801(1), 2807-d(1)(a), 2807-d(2)(c) (McKinney 1993).

The HFA tax on diagnostic and treatment centers is now fixed at "six-tenths of one percent of each facility's gross receipts received from all patient care services and other operating income on a cash basis." N.Y. PUB. HEALTH LAW § 2807-d(2)(c) (McKinney 1993). Taxable receipts include monies received from patient care services, from investment income, and from certain enumerated categories of operating income, including parking lot revenues, gift shop sales, fees for reproducing medical records, proceeds from the sale of scrap and waste, and sales rebates and commissions (App. 60-61). *See* N.Y. PUB. HEALTH LAW §§ 2807-d(3)(c), 2807-d(3)(d) (McKinney 1993). Grants, charitable contributions, donations, bequests, and government deficit financing are exempt from the tax (App. 61). *See* N.Y. PUB. HEALTH LAW § 2807-d(3)(d) (McKinney 1993). Revenues raised by the HFA are paid into the general fund of the State of New York (App. 29, 48). *See* N.Y. PUB. HEALTH LAW § 2807-d(9) (McKinney 1993 & Supp. 1996).

Fund's ERISA Challenge

The Fund filed reports and paid the HFA for the first eleven months of 1991 (App. 80-82, 84-96; C.A. App. 134-41). The Fund excluded from taxable receipts the more than $19 million it disburses to the medical centers to pay their operating expenses. None of these remittances relate to the furnishing of services for particular patient care or constitute income from the operations of the centers. These transfers are no more than the utilization of employer contributions to provide the healthcare benefits furnished by the Fund. These weekly transfers of employer contributions are akin to grants that are nontaxable under New York law. *See* N.Y. PUB. HEALTH LAW § 2807-d(3)(d) (McKinney 1993). While the State has not yet contested the exclusion from the tax of Fund remittances to the centers, *see* PB at 5; SG at 4 n.3, any future change in the State's position could expose the Fund to HFA tax liabilities of about $120,000 per year.

The Fund does not contest the HFA in its entirety, only its application to contributions or other payments for healthcare benefits provided by the Fund through its medical centers. Consequently, the Fund does not challenge imposition of the HFA on miscellaneous revenue earned by the medical centers for activities that are unrelated to the healthcare benefits provided to plan participants and beneficiaries. This miscellaneous income includes proceeds from the sale of waste or scrap, fees for reproducing medical records, and supplier rebates and commissions (App. 82).

The Fund's ERISA pre-emption challenge in this case is limited to New York's attempt to assess two categories of receipts paid directly to the Fund's medical centers:

1) Contributions to fund the healthcare benefits provided by the Fund; and

2) Payments made for benefits provided to ERISA plan participants and their beneficiaries.

The first category involves amounts mandated by the terms of applicable collective bargaining agreements for the funding

of the healthcare benefits provided by the Fund (App. 78). This category includes both employer and employee contributions that are required to be paid whether or not the beneficiaries for whom the payments are made ever receive healthcare benefits from the Fund's medical centers. The employer contributions consist entirely of man-hour contributions from non-longshore, outside employers (App. 78); the employee contributions consist of prescription charges and annual registration fees imposed on plan beneficiaries through the collective bargaining process to help defray the rising healthcare costs encountered by the Fund (App. 79).[3]

The second category encompasses payments received directly by the Fund's medical centers for benefits provided. It includes payments from the NYSA-PPGU welfare plan for healthcare benefits provided to its beneficiaries and payments received from workers' compensation insurance carriers for healthcare benefits provided to Fund participants injured on the job (App. 78-79).

The receipts in both categories, which totalled about $1.16 million for the first eleven months of 1991, were received directly by the medical centers and were included in the HFA reports as taxable amounts for which the Fund paid the HFA tax of about $7,000 (App. 80-82). The Fund has since restructured its payment procedures. Since early 1992 the registration fees and the payments from the NYSA-PPGU Welfare Fund have been remitted to the Fund, not to the medical centers (App. 82-83).

The Fund protested the HFA on ERISA pre-emption grounds (App. 82, 97-102). When the State of New York denied the protest (C.A. App. 147-48), the Fund ceased payment of the tax and commenced this lawsuit both for a declaratory judgment that ERISA pre-empts the HFA to the extent that it

[3] The prescription charges are payable only when a beneficiary actually fills a prescription at a medical center. Hence, these charges could be classified in the second category, since they are paid only when benefits are received.

taxes contributions and other payments for healthcare benefits provided by ERISA plans and for restitution of the pre-empted taxes already collected from the Fund (App. 23, 82).[4]

Decision of District Court

On cross motions for summary judgment, the district court concluded that ERISA does not pre-empt the HFA (Pet. App. 16a, 19a, 22a).[5] The district court's pre-emption analysis was deceptively simplistic. While candidly admitting that the HFA leaves less money for benefits (Pet. App. 12a), the district court considered the rate of tax not large enough to pose a "serious economic threat" to ERISA plans (Pet. App. 21a). While acknowledging that there are no numerical standards prescribed in the law to determine just how much tax is too much, the district court's own subjective barometer sensed that the HFA's 0.6% rate "is not significant enough to warrant pre-emption of a state's traditional taxing power without explicit instructions from Congress" (Pet. App. 22a).

Decisions of Court of Appeals

The court of appeals reversed (App. 10-18). It rejected the proposition advanced by the district court, "that a regulation which has a *de minimis* economic impact on ERISA plans may withstand preemption regardless of what plan operations it affects" (App. 18). In the opinion of the court of appeals, whatever the magnitude of the exaction, "an immediate tax on payments and contributions which were intended to pay for participants' medical benefits" is pre-empted by ERISA because it "'relates to' the Fund and does so in a very direct way" (App. 16). "It does not touch the Fund only slightly on

[4] The petitioners, defendants below, are the New York officials responsible for the enforcement of the HFA. For ease of reference they will be referred to in this brief as the "State."

[5] The district court did not have to decide the legal sufficiency of the State's defense under the Tax Injunction Act, 28 U.S.C. § 1341 (1994) (Pet. App. 19a), and never reached the State's Eleventh Amendment defense.

the outer limits of its plan activities; it affects the very operations and functions that make the Fund what it is, a provider of medical, surgical, and hospital care to its participants and their beneficiaries" (App. 16).

The first petition for certiorari in this case was filed a few months before the decision in *New York State Conference of Blue Cross & Blue Shield Plans v. Travelers Ins. Co.*, 115 S. Ct. 1671 (1995). When this Court remanded the case for further consideration in light of its *Travelers* decision, *see Chassin v. NYSA-ILA Medical & Clinical Serv. Fund*, 115 S. Ct. 1819 (1995), the court of appeals concluded that *Travelers* does not disturb "the ground upon which our earlier decision in this case rested" (App. 8). In the view of the court of appeals, the facts in this case distinguish it from *Travelers* because, as was found in its earlier determination, the HFA imposes "an immediate tax on payments and contributions which were intended to pay for participants' medical benefits . . . [and] thus directly affects the Fund in its principal role as an employee welfare benefit plan" (App. 9). This type of "direct economic impact on ERISA plans" is sufficient for a finding of ERISA pre-emption (App. 9).

SUMMARY OF ARGUMENT

There are certain connections between state laws and ERISA plans that are so clear and unmistakable that no prolonged and complicated analysis is necessary to conclude that these laws relate to the plans and so are pre-empted by ERISA. A state law that directly taxes contributions and other payments for healthcare benefits provided by ERISA plans falls within that category. To deny that the tax relates is to defy reality.

The careful analysis, as prescribed by this Court in *Travelers,* of Congressional will by reference to the objectives of ERISA, its legislative history, and the language of its pre-emption clause yields the same result. Congress did not intend to permit state taxation of ERISA plans. In enacting ERISA Congress rejected a request by the Executive Branch to exclude state tax laws from ERISA pre-emption. Indeed, state

taxes are expressly included within ERISA's broad pre-emption provision. True, state tax laws are pre-empted insofar as they "relate" to ERISA plans. But if a tax upon benefits and contributions — the very essence of ERISA plans — does not relate to the plans, it is difficult to conceive of a law that does. No case has ever ruled otherwise. Pre-emption of state taxation of contributions and other payments for healthcare benefits provided by ERISA plans furthers the basic purposes and objectives of ERISA: safeguarding plan assets, promoting nationally uniform administrative practices and benefit packages, and assuring use of plan assets exclusively for plan participants and their beneficiaries. Pre-emption of state taxes is consistent with the national system of tax exemption established by Congress for ERISA plans. The express Congressional design concerning state tax laws confirms what the commonsensical reading of the plain language of ERISA's pre-emption clause reveals — a state tax law, like the HFA, which directly taxes contributions and other payments for healthcare benefits provided by ERISA plans relates to the plans and is pre-empted.

The state and its supporting amici offer three principal arguments to evade pre-emption, none of which has merit. First, they hide behind the pretense that the HFA taxes the Fund not as an ERISA plan but as a hospital, or at most taxes the "benefits," not the plan. The fallacy here is that under ERISA a plan is defined as the provider of benefits. What the state and its allies call hospital functions are the very activities that make the Fund an ERISA plan. The distinction between the plan and the benefits it provides is equally specious. If the State were free to tax benefits, it could also regulate benefits, which this Court has repeatedly said it cannot do. Nor does the claim that the HFA is generally applicable to the healthcare field save it from pre-emption. The only laws of general applicability expressly exempted from ERISA pre-emption are state criminal laws. No other law of general application is afforded special treatment. Semantics cannot obscure the reality that when a state law taxes the activities which are the defining elements that qualify a plan for ERISA protection, that law relates to the plan and is pre-empted.

Second, the United States and NELA adopt a methodology that would confine pre-emption to those kinds of state laws that have already been held to be pre-empted. This categorization approach would transform ERISA's broad pre-emption clause into an extremely narrow provision. Congress has already rejected a like proposal of the Executive Branch to define pre-emption by subject matter. What the amici propose is nothing less than a petrification of the law, not an effectuation of the will of Congress.

Third, the State and its allies prophesy a general collapse of state healthcare regulation if the court of appeals is affirmed. Their alarm is unfounded. In earlier cases the Court has had occasion to comment on state laws that affect plans not as healthcare providers but in tangential roles incidental to plan activities — as purchasers, employers, lessees, etc. The Court has suggested that these kinds of laws are not pre-empted because they are too "tenuous, remote or peripheral." Laws governing the healthcare field manifestly relate to the plans *qua* plans and cannot be claimed to be tenuous, remote or peripheral. They may be saved from pre-emption on a case-by-case basis by application of a *Travelers* type analysis designed to unearth the intent of Congress. Given the presumption that Congress did not intend to pre-empt exercises of traditional state police powers, healthcare regulations may escape pre-emption. But the presumption applies only in the absence of a clear and manifest expression of Congressional intent to pre-empt. With respect to state tax laws that expression is contained in ERISA itself in a provision that expressly singles out these laws for inclusion in the pre-emption clause. The notion that by upholding Congress's express will in this case the Court would doom all state healthcare regulations is far-fetched. Far more foreboding is the likelihood that, if this Court should reverse, ERISA plans will be seen as easy targets for the states' ever burgeoning appetites for revenue.

ARGUMENT

I

ERISA PRE-EMPTS STATE LAW TO THE EXTENT IT DIRECTLY TAXES ERISA PLANS BASED UPON PAYMENTS AND CONTRIBUTIONS FOR HEALTHCARE BENEFITS FURNISHED TO PLAN PARTICIPANTS AND BENEFICIARIES BY MEDICAL CENTERS OWNED AND OPERATED BY THE PLANS

A. A Law That Taxes ERISA Plans Relates To ERISA Plans

ERISA § 514(a) provides that ERISA "shall supersede any and all State laws insofar as they may now or hereafter relate to any employee benefit plan" covered by ERISA. 29 U.S.C. § 1144(a) (1994).[6] The question in this case is whether a state law that taxes plan contributions and payments for benefits relates to an ERISA plan.[7] When a state law reaches into the coffers of an ERISA plan and extracts monies, it relates to that plan. To reach any other conclusion would do violence to the plan meaning of the language employed by Congress to express its pre-emptive intent.

This Court's decision in *Travelers* does not bode otherwise. *Travelers* did not purport to be a wholesale rewriting of the well established law of ERISA pre-emption.[8] The "relate to"

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[6] None of the specific exceptions to pre-emption delineated in the statute, *see* 29 U.S.C. § 1144(b) (1994), is applicable in this case. None has been invoked by the State.

[7] In this case the Fund does not challenge the imposition of the HFA in its entirety but only insofar as it applies to contributions and other payments for healthcare benefits provided by the Fund through its medical centers.

[8] The controlling test for determining whether a state law meets the "relate to" standard of ERISA § 514(a) is simple and direct: every state law that has a connection with or reference to an ERISA employee benefit plan relates to that plan and is pre-empted, whatever the state law's underlying

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standard is still the governing touchstone. It is still "deliberately expansive," *see Pilot Life*, 481 U.S. at 46, and "conspicuous for its breadth," *see FMC Corp.*, 498 U.S. at 58. "Relate to" still means having a connection with or reference to an ERISA plan. When the reference or connection is direct and self-evident, no further inquiry is necessary. Only when the nexus is not so clear cut, must the analysis explore all the surrounding circumstances, including the objectives of the ERISA statute, the language of its pre-emption clause, and the context of its legislative passage, to discern the type of state

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purpose and intent. *District of Columbia v. Greater Wash. Bd. of Trade*, 506 U.S. 125, 129-130 (1992); *FMC Corp. v. Holliday*, 498 U.S. 52, 58-59 (1990); *Shaw v. Delta Air Lines*, 463 U.S. 85, 96-97 (1983). The "relate to" standard must be interpreted broadly, *see Shaw*, 463 U.S. at 98; it must be given a "broad common sense meaning," *see Ingersoll-Rand Co. v. McClendon*, 498 U.S. 133, 139 (1990). The "relate to" touchstone does not limit ERISA pre-emption only to state laws specifically designed to regulate the terms and conditions of employee benefit plans or those dealing with the subjects of regulatory concern covered by ERISA — reporting, disclosure, fiduciary responsibility, and the like. *Ingersoll-Rand*, 498 U.S. at 137-39; *Shaw*, 463 U.S. at 98. Pre-emption prevails even if the state law is not specifically designed to affect ERISA plans or the state law's connection, reference, or effect is only indirect, *see Greater Wash. Bd. of Trade*, 506 U.S. at 129-130; *Ingersoll-Rand*, 498 U.S. at 139; *Pilot Life Ins. Co. v. Dedeaux*, 481 U.S. 41, 47-48 (1987), or the state law is "consistent with ERISA's substantive requirements," *see Metropolitan Life Ins. Co. v. Massachusetts*, 471 U.S. 724, 739 (1985). Nor does a state law automatically escape ERISA's pre-emptive reach merely because it is a general law not specifically designed to affect employee benefit plans. *Shaw*, 463 U.S. at 96-98. If the state law relates to an employee benefit plan, it is pre-empted regardless of its status as a law of general application. *Morgan Guar. Trust Co. v. Tax Appeals Tribunal*, 80 N.Y.2d 44, 49, 587 N.Y.S.2d 252, 255, 599 N.E.2d 656 (1992). "Even in the absence of an express link to an employee benefit plan, State law is preempted 'insofar as' the law applies to benefit plans in particular cases." *Morgan Guar.*, 80 N.Y.2d at 49, 587 N.Y.S.2d at 255 (citing *Shaw*, 463 U.S. at 97 n. 17). Only when the state law's connection to the ERISA plan is "too tenuous, remote, or peripheral," might the state law be saved from ERISA's pre-emptive grasp, *see Shaw*, 463 U.S. at 100 n.21, "as is the case with many laws of general applicability," *see Greater Wash. Bd. of Trade*, 506 U.S. at 130.

law Congress intended to displace. *Travelers*, 115 S. Ct. at 1677, 1680.

This Court's prior ERISA pre-emption cases conform to this approach. In *Mackey v. Lanier Collection Agency & Serv., Inc.*,486 U.S. 825 (1988), it took the Court, with no objection by the four dissenting justices, *id.* at 841, less than two pages to find a provision of the Georgia garnishment law that expressly exempted ERISA plans pre-empted because "[t]he state statute's express reference to ERISA plans suffices to bring it within the federal law's preemptive reach." *Id.* at 830. The Court's later decision in *Greater Wash. Bd. of Trade* reaffirmed the principle that a state law having a specific reference to ERISA plans "on that basis alone is pre-empted." 506 U.S. at 130.

The lesson of *Mackey* and *Greater Wash. Bd. of Trade* is not limited to those cases involving state laws that contain specific reference to ERISA plans. Obviously, when the language of the state statute refers expressly and directly to ERISA plans, there is no question that the state law satisfies the "reference to" prong of the "relate to" test. A state law may also satisfy the "connection with" prong in a similarly unmistakable fashion when it applies manifestly to ERISA plans. *See Metropolitan Life Ins. Co. v. Massachusetts*, 471 U.S. 724, 739 (1985) (state law requiring all health insurance contracts to provide minimum mental healthcare benefits indisputably relates to insured ERISA plans but is exempted from pre-emption by insurance savings clause); *Alessi v. Raybestos-Manhattan, Inc.*,451 U.S. 504, 524-25 (1981) (anti-set-off provision of workers' compensation law relates to ERISA plans because it prohibits one method for calculating benefits that is permitted by federal law). Certain state laws have so unmistakable a connection with ERISA plans that no honest appraisal can conclude that they do not relate. When the ball flies over the center field fence, the umpire does not have to refer to the foul poles. *See, e.g., Alessi*, 451 U.S. at 525 (relationship of state law to ERISA plans so clear that there was no need to "determine the outer bounds of ERISA's

pre-emptive language to find this New Jersey provision an impermissible intrusion on the federal regulatory scheme").

When the state law applies to ERISA plans indirectly, the pre-emption analysis becomes more complicated. In *Mackey,* when the Court addressed the less pellucid issue of whether that portion of the Georgia garnishment statute which contained no direct reference to ERISA plans was pre-empted, a far more expansive analysis ensued, provoking a sharp division in the Court. To ascertain whether a law of indirect application relates to ERISA plans requires the kind of analysis engaged in by the *Travelers* Court. A state law that directly taxes ERISA plans does not.

Unless the tax law relates, the ERISA plan would not have to pay the tax. It is true that a tax law may have a connection to an ERISA plan that does not rise to the level of actually imposing an exaction on the plan itself. *See, e.g., Thiokol Corp. v. Roberts,* 76 F.3d 751 (6th Cir. 1996), *petition for cert. pending,* No. 95-1913 (U.S. filed May 23, 1996) (business tax upon employer based in part upon its contributions to ERISA plans held not pre-empted). In those situations a *Travelers* analysis may be appropriate. But when the state enactment reaches into the plan's coffers to siphon off monies, the connection is so direct and unmistakable as to leave no doubt that it relates to the plan. Let the State and its allies tell the taxpayers on April 15 that the Internal Revenue Code does not relate to them.

B. Even Under A *Travelers* Analysis The New York Tax Is Pre-empted

Application of the methodology employed by this Court in *Travelers* leads to the same conclusion, that the HFA relates to ERISA plans. The *Travelers* Court underscored the primacy of Congressional intent in determining what state laws are supplanted by ERISA pre-emption, and Congress has left no doubt that it did not intend to exempt state tax laws of general applicability from ERISA's pre-emption provision.

When ERISA was first under legislative consideration in 1974, Congress rejected a request by the Secretaries of Labor and Treasury to exclude state tax laws from ERISA pre-emption. *Northwest Airlines v. Roemer,* 603 F. Supp. 7, 12 (D. Minn. 1984). In 1983 Congress added a specific provision reaffirming that ERISA's pre-emption clause applied to state tax laws. This provision contains a clear and unambiguous statement of Congressional intent to include within ERISA's pre-emptive embrace "any State tax law relating to employee benefit plans." 29 U.S.C. § 1144(b)(5)(B)(I) (1994). The conference report explained that this statutory directive assured that "preemption is continued with respect to . . . any state tax law relating to employee benefit plans . . . ." H.R. CONF. REP. No. 984, 97th Cong., 2d Sess. 18 (1982), *reprinted in* 1982 U.S.C.C.A.N. 4603.

This addition to ERISA's pre-emption clause was enacted as part of a legislative package containing a Congressional dispensation for Hawaii's Prepaid Health Care Act that expressly saved that state enactment from ERISA's pre-emptive reach. *See generally Retirement Fund Trust of Plumbing v. Franchise Tax Bd.,* 909 F.2d 1266, 1277-78 (9th Cir. 1990) (explaining the legislative history of the special exemption afforded Hawaii's healthcare legislation under ERISA). In introducing the special Hawaiian legislation on the Senate floor, Senator Dole said:

> The bill amends ERISA to provide that Hawaii law relating to employer maintained health insurance plans would not be preempted by ERISA to the extent that the Hawaiian law does not relate to matters thoroughly regulated under ERISA or *impose tax liability on insurance premiums or benefits.*

128 CONG. REC. S26902 (daily ed. Oct. 1, 1982) (emphasis added).

There is a clear and unequivocal Congressional policy to shield ERISA plans from the destructive effects of taxation. Congress has meticulously put into place throughout the

Internal Revenue Code a national system of tax exemptions for ERISA plans. Contributions to ERISA plans are deductible and tax exempt. *See, e.g.*, 26 U.S.C. § 404 (1994) (pension and annuity plans); 26 U.S.C. §§ 162, 419, 419A (1994 & Supp. I 1995) (welfare benefit plans); 26 U.S.C. § 79 (1994) (employer-paid cost of life insurance); 26 U.S.C. § 106 (1994) (contributions to disability plans). Income earned through the investment of plans' assets is tax-free. *See, e.g.*, 26 U.S.C. §§ 501(a), 501(c) (1994) (pension and welfare plans). Benefits received from welfare plans are tax-exempt. *See* 26 U.S.C. §§ 104, 105, 106 (1994) (accident and health benefits); 26 U.S.C. § 101 (1994) (death benefits); 26 U.S.C. § 129 (1994) (dependent care benefits). Benefits received from pension plans are tax-deferred; pension benefits are nontaxable while held by ERISA plans and only become taxable when ultimately received by the beneficiary. *See* 26 U.S.C. §§ 402, 403 (1994). ERISA's pre-emption clause preserves this uniform, nationwide system of tax exemptions from the unsettling effects of conflicting state tax policies.

The court of appeals in this case applied the ordinary, commonsensical meaning of the statutory "relate to" standard and found that a state tax assessment levied upon employer and employee contributions to a medical center operated by an ERISA welfare plan to provide medical benefits to plan beneficiaries has a direct connection to the plan and is pre-empted, not in its entirety but only insofar as it assesses those contributions. Indeed, every court that has addressed a state tax law that directly assesses plan contributions or plan benefits has found that the state enactment relates to ERISA plans and is pre-empted. *E-Systems, Inc. v. Pogue*, 929 F.2d 1100, 1103 (5th Cir.), *cert. denied*, 502 U.S. 981 (1991) (tax on healthcare benefits paid by plan); *General Motors Corp. v. California State Bd. of Equalization*, 815 F.2d 1305, 1309 (9th Cir. 1987), *cert. denied*, 485 U.S. 941 (1988) (tax computed on basis of healthcare benefits paid by plan); *Birdsong v. Olson*, 708 F. Supp. 792, 798-799, 801 (W.D. Tex. 1989), *appeal dismissed*, 901 F.2d 1270 (5th Cir. 1990) (tax on healthcare benefits paid by plan); *National Carriers' Conference Comm. v. Heffernan*, 454 F. Supp. 914, 917-18 (D. Conn. 1978) (same).[9]

Congress expressly included within the scope of ERISA's pre-emption clause state tax exactions. Anything less would have encouraged the various states to impose their individual tax levies upon the treasuries of ERISA plans. "The cost of the plan must therefore increase for the employer and/or employees or the benefits must be adjusted downwards to offset the tax bite. This is the type of impact Congress intended to avoid when it enacted the ERISA legislation." *E-Systems*, 929 F.2d at 1103; *see also Northwest Airlines*, 603 F. Supp. at 12; *National Carriers' Conference Comm.*, 454 F. Supp. at 917.[10]

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[9] The only state tax laws that have survived ERISA pre-emption are business taxes imposed upon employers that include in the tax base contributions paid by employers to ERISA employee benefit plans, *see Thiokol Corp. v. Roberts*, 76 F.3d 751, 753-56 (6th Cir. 1996), *petition for cert. pending*, No. 95-1913 (U.S. filed May 23, 1996), and withholding and tax delinquency-garnishment procedures that assess not the plan but the beneficiary, *see Retirement Fund Trust of Plumbing v. Franchise Tax Bd.*, 909 F.2d 1266, 1281 (9th Cir. 1990); *Firestone Tire & Rubber Co. v. Neusser*, 810 F.2d 550, 556 (6th Cir. 1987); *see also Mackey v. Lanier Collection Agency & Serv., Inc.*, 486 U.S. 825 (1988) (monies subject to state garnishment had already been earmarked for judgment debtor so that application of state garnishment law did not affect amount of plan benefits due other beneficiaries); *Aetna Life Ins. Co. v. Borges*, 869 F.2d 142 (2d Cir.), *cert. denied*, 493 U.S. 811 (1989) (same result for state escheat law). None of these state tax laws had any effect upon the amount of plan benefits or contributions, and thus their connection to ERISA plans was tenuous, remote, and peripheral — an insufficient nexus to trigger pre-emption. *Thiokol*, 76 F.3d at 754-55.

[10] The HFA unquestionably increases the cost to the Fund of providing beneficiaries with a given level of healthcare benefits. The outside employers' man-hour contributions are effectively lowered by the amount of the assessment. Similarly, the NYSA-PPGU Welfare Plan's payments, which are essentially benefit payments for its own beneficiaries, must be increased so that the Fund and its medical centers can recoup the added costs caused by the HFA assessment. The state levy also disrupts the balance struck between employer contributions and employee prescription and registration payments, reducing the effectiveness of these employee contributions in helping to defray healthcare costs. Obviously, if the state

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The Congressional treatment of state tax laws within the context of ERISA pre-emption exposes the fallacy of the interpretation of *Travelers* propounded by the State and its amici supporters. *See* PB at 14; HA at 4. *Travelers* does not, as they suggest, stand for the proposition that state laws whose only impact on ERISA plans is economic are saved from pre-emption. First, there is nothing in the *Travelers* decision that supports this reading. The ruling of the *Travelers* Court was limited to indirect economic impacts of an insubstantial nature. *See* 115 S. Ct. at 1683. Second, tax laws by their very nature have as their necessary effect a purely economic impact on the affected taxpayer. If economic impact alone could never trigger pre-emption, the explicit Congressional directive to include state tax laws that relate to ERISA plans within the reach of ERISA pre-emption is rendered meaningless. Adoption of the suggestion urged by the State and its allies would effectively read the directive out of the statute, a result that would violate basic principles of statutory construction. *See Travelers*, 115 S. Ct. at 1679-80.

To shield state tax enactments from ERISA pre-emption would be inconsistent with the basic "objectives of the ERISA statute" — a key guide to defining the scope of ERISA pre-emption. *Id.* at 1677.[11] Subjecting ERISA plans to the taxing

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levy increases the cost of medical and related healthcare services, ERISA plans must reduce benefits to stay within their budgetary restrictions or charge their beneficiaries more for the benefits unless the union representatives of the plan participants prevail in persuading their employers at the bargaining table to increase the level of employer contributions.

[11] The function of federal pre-emption is to give effect to the "full purposes and objectives" of federal legislation, *see Hines v. Davidowitz*, 312 U.S. 52, 67-68 (1941), since, under the Supremacy Clause of the Constitution of the United States, federal law is "the supreme Law of the Land" to which "any Thing in the Constitution or Laws of any State to the Contrary" must yield. U.S. CONST., ART. VI, CL. 2. In determining whether federal law pre-empts state law, "[t]he purpose of Congress is the ultimate touchstone." *Malone v. White Motor Corp.*, 435 U.S. 497, 504 (1978) (quoting *Retail Clerks v. Schermerhorn*, 375 U.S. 96, 103 (1963)); *Pilot Life Ins. Co. v. Dedeaux*, 481 U.S. 41, 45 (1987). When Congress adopts a broadly worded express pre-emption provision in the federal legislation, *(Footnote continued)*

powers of 50 separate and distinct jurisdictions would frustrate the Congressional objective of allowing ERISA plans to adopt nationally uniform administrative practices and interstate benefit packages, which the *Travelers* Court identified as one of the principal goals of ERISA pre-emption. *See Travelers*, 115 S. Ct. at 1677-78. State taxation of plan benefits and plan contributions would be incompatible with this Congressional objective. Benefits would vary from state to state depending upon the magnitude of each state's tax bite. This Congressional objective is implicated in this very case.

The Fund had to restructure its payment procedures in response to the state exaction. Since July 1992 registration fees and payments from the NYSA-PPGU Welfare Fund have been remitted to the Fund, not to the medical centers. Although the Fund can redirect to the Fund all payments now received by the medical centers and thus effectively override the HFA, it has not yet done so. Incentives that induce or encourage changes in plans' traditional methods of administration are features of state regulation that ERISA pre-emption was designed to prevent. *See National Carriers' Conference Comm. v. Heffernan*, 454 F. Supp. 914, 918 (D. Conn. 1978). When the affected ERISA plan provides multi-state coverage, as does the Fund here, the state scheme has the effect of disrupting uniform administrative procedures that apply across state lines. The New York tax at issue in this case has no New Jersey counterpart and would require the Fund to restructure its established method of administration to implement procedures for its New York medical clinics that differ from those of its New Jersey clinic (App. 17). Any state law that forces a plan to restructure its administrative procedures or to adopt different methods on a state-by-state basis relates to an ERISA plan and is pre-empted. *FMC Corp.*, 498 U.S. at 59-60; *Alessi*, 451 U.S. at 524.

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the "task of discerning congressional intent is considerably simplified." *Ingersoll-Rand Co. v. McClendon*, 498 U.S. 133, 138 (1990).

To shield state tax enactments from ERISA pre-emption would also be inconsistent with the Congressional objective of safeguarding plan assets. ERISA was promulgated principally to protect the interests of plan participants by assuring "the soundness and stability of plans with respect to adequate funds to pay promised benefits." 29 U.S.C. § 1001(a) (1994); *see Northeast Dep't ILGWU Health & Welfare Fund v. Teamsters Local 229 Welfare Fund*, 764 F.2d 147, 158 (3d Cir. 1985) ("Resolution of the issue will affect not only the interests of beneficiaries, whose welfare it was the prime purpose of ERISA to protect, but also the interests of benefit trust funds, the solvency of which was a matter of serious congressional concern").

To shield state tax enactments from ERISA pre-emption would also be inconsistent with a third Congressional objective. ERISA requires that employee benefit plans expend plan funds solely for the benefit of plan participants and their beneficiaries. 29 U.S.C. § 1103(c)(1) (1994). Indeed, it would be a violation of § 302 of the Labor Management Relations Act, 29 U.S.C. § 186 (1994 & Supp. I 1995), to use trust assets to benefit persons other than plan beneficiaries. *See In re Typo-Publishers Outside Tape Fund*, 478 F.2d 374, 375-76 (2d Cir.), *cert. denied*, 414 U.S. 1002 (1973); *Blassie v. Kroger Co.*, 345 F.2d 58, 68 (8th Cir. 1965). Even if trustees wanted to donate plan money to help the indigent, ERISA's exclusive benefit requirement would prohibit them from doing so. Any state law that mandates this ERISA violation would clearly relate to ERISA plans because it effectively enrolls strangers on the roster of plan beneficiaries. *Bricklayers Local 1 Welfare Fund v. Louisiana Health Ins. Ass'n*, 771 F. Supp. 771, 775 (E.D. La. 1991) (ERISA preempts use of assets of ERISA plans to establish fund to finance hospital care for those suffering from catastrophic illnesses). How much more violative of Congress's purpose is a state law that expropriates plan assets for the benefit of the general public.

To sacrifice plan solvency, uniformity, and exclusivity to the appetite of state tax collectors would contravene the very ERISA purposes Congress had in mind when it included state

tax laws within the reach of ERISA's pre-emption clause. In light of this express Congressional design, which the *Travelers* Court considered to be of decisive importance in pre-emption analysis, it is difficult to find a state law that more clearly relates to ERISA plans than one which directly taxes contributions and payments for benefits provided to plan participants and their beneficiaries. The *Travelers* analysis thus confirms what the commonsensical reading of the plain language of ERISA's pre-emption clause reveals — that the HFA relates to ERISA plans and is pre-empted.

II

AN ERISA PLAN AND ITS OPERATIONS CANNOT BE BIFURCATED TO EVADE PRE-EMPTION

The main argument propounded by the State and its amici is an exercise in semantic manipulation. They claim that the HFA is not pre-empted because it is imposed upon the Fund in its capacity as a "provider of medical services," not as a "plan." *See* PB at 16; SG at 25; HA at 19; NELA at 19; AG at 12, 20 n.8. This ontological argument is premised upon a fundamental error. The State and its allies pretend that ERISA employee welfare benefit plans are not themselves the providers of healthcare benefits but exist only as self-insurers or purchasers of healthcare insurance that will be used to pay the actual healthcare providers. Somehow the definition adopted by Congress has been lost in the shuffle.

An employee welfare benefit plan is "any plan, fund, or program . . . established or . . . maintained for the purpose of providing for its participants or their beneficiaries, through the purchase of insurance or otherwise," healthcare and other benefits. 29 U.S.C. § 1002(1) (1994). An ERISA plan, therefore, is not merely an insurer. It provides healthcare benefits. This may be done by paying beneficiaries' healthcare bills or by reimbursing the beneficiaries or by obtaining outside, commercial insurance. It can also be done "otherwise," *i.e.*,

by furnishing medical, surgical, or hospital care directly through clinics and other healthcare facilities.[12] In dispensing care and treatment at its clinics the Fund is engaging in the very activity that brings it within the ERISA definition of an employee welfare benefit plan.

To pretend that an ERISA plan enjoys a hypostatic union in which its two natures coexist simultaneously without commingling borders on the nonsensical. It is equally fatuous to say that the Fund "chooses" to act as a hospital. *See* SG at 25; AFSCME at 4. It is an essential nature of the Fund to act as the direct provider of medical care. What the State and its allies call hospital functions are the very activities that make the Fund an ERISA plan.

The State buttresses its argument by suggesting that the medical centers operated by the Fund are essentially no different than any general, commercial facility. *See* PB at 16; *see also* AFSCME at 13. This paints a false picture. The Fund's clinics are not commercial enterprises. They are not open to the general public. They are not operated for profit. They cannot pass on the HFA or other state taxes to their "customers." They do not compete with other healthcare facilities for their clientele. They exist purely and simply to provide ERISA welfare benefits to plan participants and their beneficiaries. Recognition of the true nature of the Fund's activities refutes the assumption that the Fund is similarly situated to other healthcare providers and should be similarly treated. *See* SG at 25.[13]

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[12] The United States and other amici recognize that ERISA plans are increasingly electing to provide benefits directly. *See* HA at 9-10 & n.6 (day care centers and clinics); AG at 9 (day care centers, legal services programs, and medical centers); SG at 29 (day care centers and funeral homes).

[13] The Fund's clinics are more akin to the kinds of institutions which the New York statute itself exempts from the tax: voluntary nonprofit hospitals totally financed by charitable contributions, facilities dedicated solely to the care of police, firefighters, and emergency service personnel, etc. N.Y. PUB. HEALTH LAW § 2807-d(b) (McKinney 1993). Of course, the

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A leitmotif that trills throughout the briefs of the State and its allies is the claim that the HFA is a law of "general application." *See* PB at 11; SG at 25; AG at 12-13; NGA at 8; AFSCME at 13- 17. The observation offers little solace. General applicability is not a talisman that magically shields state laws from ERISA pre-emption. General applicability *per se* is insufficient to avoid pre-emption; a state law does not have to target an employee benefit plan directly in order to be pre-empted by ERISA, *see Pilot Life*, 481 U.S. at 47. The very language of ERISA's pre-emption clause dispels the notion that Congress reserved for state laws of general applicability a special status that renders them less susceptible to pre-emption. The only mention of a state law of general application appears in ERISA § 514(b)(4), which exempts from ERISA pre-emption "any generally applicable *criminal* law of a State." 29 U.S.C. § 1144(b)(4) (1994) (emphasis added). No other generic law of general application is afforded special treatment.[14]

State tax laws, which sweep broadly and usually apply to the entire taxpaying public, were explicitly singled out by Congress to assure that there would be no misunderstanding that they are subject to ERISA's express pre-emption clause regardless of their general applicability. HFA, then, is not any less susceptible to ERISA pre-emption merely because it happens to be a state tax. The language of ERISA's pre-emption clause as well as its legislative history indicate that Congress intended to pre-empt state tax laws of general applicability to

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New York Legislature could not add ERISA plan facilities to the list of exemptions without incurring automatic pre-emption under the rule in *Mackey*.

[14] Because of the categorical application of ERISA's pre-emption clause to state laws of general application, only Congress can save specific state enactments from ERISA's pre-emptive reach. Hawaii's Prepaid Health Care Act, generally applicable to all employers in that state, would have been pre-empted by ERISA despite its status as a law of general application, were it not for an express Congressional dispensation. *See* 29 U.S.C. § 1144(b)(5) (1994).

the same extent as other laws insofar as they relate to ERISA employee benefit plans.

The HFA's tax on contributions and payments for benefits affects ERISA plans *qua* employee benefit plans; it goes to the very activities and functions that make employee benefit plans what they are. As the Second Circuit observed, "It does not touch the Fund only slightly on the outer limits of its plan activities; it affects the very operations and functions that make the Fund what it is, a provider of medical, surgical, and hospital care to its participants and their beneficiaries" (App. 16). It is hardly remarkable that Congress intended the express pre-emption clause of ERISA to proscribe state assessments that sap the lifeblood of ERISA plans and directly hinder their essential functions.

The State proffers a second semantic gyration, a variation of its ontological argument. It claims that "ERISA is concerned with the administrative structure established to provide the benefits, not the benefits themselves" and that the state law "relates, at most, to the benefit provided by the plan, not to the plan itself." *See* PB at 23-24. The State in essence would have it that the only kind of connection that would trigger pre-emption is where the state law would interfere with the administration of the plan. *See* PB at 23-25; *see also* HA at 16-19. Manifestly, ERISA plans exist for the purpose of providing benefits. All the structure and administration of the plan is geared to that end. ERISA plans do not provide benefits so that they may engage in administration; they are administered so that they may provide benefits. The State's suggestion that laws which interfere with administration are pre-empted but laws that interfere with the ability to provide benefits are not turns ERISA pre-emption on its head. The State's position would permit the states to regulate ERISA benefits without incurring pre-emption, a position directly at odds with prevailing law. *See Shaw*, 463 U.S. at 97 (state law mandating pregnancy benefits is pre-empted); *Metropolitan Life*, 471 U.S. at 730 n.11, 735 n.14 (state law mandating minimum mental health benefits would have been pre-empted but for insurance savings clause).

Semantics cannot obscure the reality that when a state law taxes the activities which are the defining elements that qualify a plan for ERISA protection, that law relates to the plan and is pre-empted.

III

ERISA PRE-EMPTION IS NOT CONFINED TO THE KINDS OF STATE LAW THAT HAVE ALREADY BEEN ADDRESSED BY THE COURT

The United States and NELA purport to provide the Court with a calculus for determining what state laws ERISA pre-empts. The United States distills from the Court's prior decisions three types of laws that are likely to be pre-empted — those which pertain to subjects already regulated by ERISA, *see* SG at 12-14, those which specifically refer to ERISA plans, *see id.* at 14-16, and those which mandate or prohibit benefits provided by ERISA plans, *see id.* at 17-19. The United States then proposes a fourth scenario in which it claims there is no pre-emption, *see id.* 19-23 — a situation that just happens to match the characterization of the facts in this case urged by the State and its cohorts. This last item does not rightly belong in the series. It is not parallel to the other three. It is not a scholarly synthesis of legal precedent but an advocate's argument. Its derivation is not what the Court has decided in the past but what forces aligned with the State hope the Court will rule in this case.

One should not be deceived by the Government's bait and switch. The fact that it tags its argument onto a series of legal distillations does not elevate it to their level. The Government's first three dissertations have nothing to do with this case. The HFA is not the kind of state law they describe, and the Fund does not urge pre-emption on those grounds. The Government's argument must stand or fall on its own merits and does not deserve the halo effect the Government seeks to obtain by its strategic placement.

Nor should one be deceived by the Government's *sotto voce* suggestion that unless a state law falls into one of the first three categories, it must necessarily fall into the fourth and thus escape pre-emption. This theme is given full rendition by another amicus. NELA constructs five situations when ERISA pre-empts state laws[15] and sets them against a presumption that a state law in an area of traditional state regulation is not pre-empted. NELA then argues that the New York law in this case escapes pre-emption because it is not one of the five pre-empted types. *See* NELA at 20.

This approach converts adjudication into a process of categorization. Prior pre-emption cases are divided into two types: those in which the state law was pre-empted and those in which the state law survived the pre-emption challenge. The Government and NELA jump to the conclusion that, if a state law does not fall within the first category, it necessarily falls within the second.

By developing principles that impose boundaries beyond which pre-emption will not extend, the approach of synthesizing existing case law circumscribes the conspicuously broad ERISA pre-emption clause to those specific situations where courts have already found pre-emption. Under this approach a state law would relate to an ERISA plan only if it resembled another state law that a prior court decision found pre-empted. It is not a judicial function to erect fences where the legislature has not. Existing case law has hardly exhausted all the possible ways in which state statutes could relate to ERISA plans. To limit pre-emption to what has already been decided is to transform an extremely broad pre-emption provision into a very narrow one. This formulation parallels that of the Executive Branch, which had sought to define ERISA

[15] According to NELA, pre-emption is triggered when the state law (1) directly conflicts with ERISA, *see* NELA at 11, (2) involves a claim that may be brought under ERISA, *see id.* at 12, (3) specifically refers to an ERISA plan, *see id.* at 13, (4) regulates an ERISA-governed relationship, *see id.* at 15, and (5) accomplishes through a severe indirect economic effect what it could not do directly, *see id.* at 19.

pre-emption by example or subject matter — an approach already rejected by Congress, *see Shaw*, 463 U.S. at 98 n. 19, and one that would lead not to the accomplishment of the legislative will but to the petrification of the law.

Another fallacy in the categorization approach is the setting up in contradistinction to state laws that have been pre-empted state laws involving areas of "traditional state regulation." It cannot be denied that this case arises within the area of healthcare or that that area is one of traditional state regulation. The error is that the state law in this case is not the regulation of healthcare. It is the imposition of a tax.

Taxation is often used as a means of regulation, such as protective tariffs or prohibitive excises on the sale of dangerous weapons. But the New York law in this case has no underlying regulatory motivation. It is a revenue raising measure (App. 32, 48; *see also* SG at 2). It has no other purpose but to swell the coffers of the State's treasury.[16]

The Fund has never claimed that it is immune from genuine state regulatory laws, like those governing the staffing and operations of medical facilities, *see* PB at 20. As to these

[16] The distinction between a regulation and a tax has already made an appearance in the jurisprudence of ERISA pre-emption. When the experimental prototype of Hawaii's prepaid healthcare law was challenged under ERISA pre-emption, one of Hawaii's defenses was that the law was an exercise of its sovereign taxing authority, immune from pre-emption by the Tenth Amendment. The Ninth Circuit rejected the defense, holding that the law was a regulation, not a tax, because any imposition did not find its way into the sovereign's purse. *Standard Oil Co. of Cal. v. Agsalud*, 633 F.2d 760, 765 (9th Cir. 1980), *aff'd*, 454 U.S. 801 (1981). The issue evaporated when this Court in *Garcia v. San Antonio Metropolitan Transit Auth.*, 469 U.S. 528 (1985), overruled *National League of Cities v. Usery*, 426 U.S. 833 (1976), the case on which Hawaii's sovereignty analysis rested. *See Retirement Fund Trust of Plumbing v. Franchise Tax Bd.*, 909 F.2d 1266, 1277 n. 43 (9th Cir. 1990). What did not disappear was the distinction between taxation and regulation, and Congress in its later dispensation for the Hawaii healthcare legislation felt constrained to add to ERISA the provision expressly including state tax laws within the ambit of the pre-emption clause. *See id.* at 1277-78.

regulatory measures the presumption that Congress does not lightly interfere with the historic police powers of the states may apply. That the presumption applies to the state's revenue raising measures is far less certain. But even if the presumption did apply, it fails in the presence of a clear and manifest Congressional intent to supplant the state laws. *Travelers,* 115 S. Ct. at 1676. ERISA contains an express declaration that state tax laws enjoy no charmed existence from ERISA pre-emption. If they relate to ERISA plans, they are pre-empted.

IV

PRE-EMPTION OF A DIRECT TAX ON ERISA PLANS DOES NOT THREATEN ALL STATE HEALTHCARE REGULATION

Ultimately the State and its allies take refuge in prophesies of gloom and doom. They predict that if this Court rules that the Fund is not subject to the New York HFA, it will mean that ERISA plans will be free of a myriad of state regulations affecting all kinds of benefits provided by ERISA plans from healthcare to legal services. *See* PB at 20; SG at 29; AG at 6-10; HA at 14-16; AFSCME at 11, 13; NGA at 15-16. They conjure up images of unqualified nurses, unprotected children, unethical lawyers, and unembalmed bodies if the decision of the court of appeals is reversed. Some even foretell of a mass emancipation from state regulation of virtually all healthcare providers through the device of transforming themselves into ERISA plans. *See* HA at 9-11.

The "tenuous, remote or peripheral" standard, *see Shaw,* 463 U.S. at 100 n. 21, disposes of most types of state regulation. The focus of this standard is the nature of the state law's effect on ERISA plans. When the state regulation touches the plan only slightly on the outer limits of its activities, the state law's connection to the plan may well be too tangential to satisfy the "relate to" test that triggers pre-emption.

The "tenuous, remote or peripheral" standard assures that the Congressional objective embodied in ERISA's

pre-emption clause is properly effectuated. Pre-emption is designed to proscribe state regulation when the state law relates to an ERISA plan *qua* an employee benefit plan, *i.e.,* when the state law affects the very activities and functions that make the employee benefit plan what it is. *Fort Halifax Packing Co. v. Coyne,* 482 U.S. 1, 7 (1987) ("ERISA's pre-emption provision does not refer to state laws relating to 'employee benefits', but to state laws relating to 'employee benefit *plans*'" (emphasis in original)).

Obviously, Congress could not have intended to shield plans from the normal incidents of state regulation that affect private enterprises generally. ERISA plans do not operate in a vacuum. A necessary incident of their activities requires them to perform functions that are not peculiar to employee benefit plans but are carried on by other private entities. ERISA plans hire employees, rent office space, and purchase goods and services. Congress could not have intended ERISA's pre-emption clause to foreclose state regulation of employee benefit plans when they act not as plans but as employers, lessees, or purchasers. *Cf. Mackey,* 486 U.S. at 833 (ERISA plans are not relieved from state law liability for unpaid rents, failure to pay creditors, or tortious conduct). By the same token state taxes and assessments that do not implicate the administration of employee benefit plans but arise as a result of transactions which any other enterprise might engage in do not trigger pre-emption because they are too remote, tenuous, or peripheral. Sales taxes, utility taxes, and gasoline taxes are some of the state levies that would fall into this non-pre-empted category.

The only real problem arises with respect to those state regulations directly involving healthcare: staffing requirements, nursing and physician standards, sanitary regulations involving disposal of medical waste, etc. These provisions which apply to plans as the providers of healthcare benefits are affecting the plans *qua* plans. It does not take Jamesian metaphysics to see that they relate to ERISA plans. *Travelers,* 115 S. Ct. at 1677. Nor can one honestly say that the

relationship is tenuous, remote or peripheral, since it goes to the essence of the plans' ERISA status.

It is precisely in this area that the Court's decision in *Travelers* offers the solution. The touchstone is the intent of Congress. The starting point of that analysis is the presumption that the exercise of historic state police powers is not pre-empted. *Travelers* observed that there was no indication of a clear Congressional intent in ERISA to pre-empt traditional state healthcare regulation. None of these regulatory measures is specifically mentioned in ERISA's pre-emption provision. It would appear, therefore, that the regulations about which the State and its allies are so concerned would survive. But that is not this case. This case involves not healthcare regulation but taxation, and as to taxation the ERISA pre-emption clause does contain an explicit expression of Congressional intent to pre-empt. As to the other types of regulation, which are the subject of the doomsaying of the State and its allies, the question of their susceptibility to ERISA pre-emption must be decided on a case-by-case basis. What is important here, however, is that it does not necessarily follow, that, if this Court invalidates a state tax, which Congress expressly included within the ambit of ERISA pre-emption, on the very activities that make the plan a plan, all peripheral state regulation of plan activities must also fall.

A far more realistic prediction of mischief that might follow upon a decision of this Court arises if the Second Circuit's decision should be reversed. In that event the states will have been given the green light to directly tax the contributions and benefits of ERISA plan. If the arguments of the State and its allies are accepted, it is difficult to conceive of any state tax that would be pre-empted. After all, state taxes, especially income taxes, are laws of general applicability involving traditional exercises of state police power that affect plans not as plans but as taxpayers. The states' appetites for money are without limit. ERISA plans provide an available fatted calf. Ominous stirrings already appear in the *amicus* brief to this Court filed on behalf of more than half the states of the Union. *See* AG at 12-14.

One can take no comfort from either the low rate of the HFA or the modest amount of the tax bite at issue. If there is anything so sure as death and taxes, it is that tax rates will rise. One need only remember the original levies under the Sixteenth Amendment. Moreover, the rate alone does not determine the amount of a tax. Definitions of the taxable transaction or tax base and of deductible items are often more instrumental in establishing the severity of the tax bite. During the pendency of this litigation the State has seen fit not to challenge the Fund's position that payment to medical centers to meet operating budgets are not subject to the HFA. Should the State prevail in this court, there is nothing to prevent it from expanding its definition of receipts to include the Fund's entire $20 million annual budget, yielding a tax of more than $120,000 per year.

Even the $7,000 figure now at issue has a real and substantial effect on the Fund's ability to provide benefits. Unlike businesses, the Fund cannot pass the tax on to its customers. Any HFA assessment must come out of the finite pool of monies contributed to the Fund. No matter how small the levy, there will be that much less to pay for plan benefits. To the individual participant who has to dig into his own pocket when the Fund is unable to provide free treatment, the hardship is considerable.

CONCLUSION

The court of appeals in this case, as did every other tribunal that has entertained the same question, applied established precedent to invalidate the immediate and direct imposition of a state tax levy on contributions to, and benefit payments of, an ERISA plan. The decision below is perfectly consistent with the analytical methodology adopted by this Court in *Travelers,* since it gives effect to Congressional intent, the touchstone for ERISA pre-emption. It rests squarely upon a clear and explicit Congressional policy to pre-empt state tax impositions on ERISA plan contributions and benefits. The decision below, therefore, is in perfect harmony with established principles of ERISA pre-emption law and should not be disturbed. The judgment of the court of appeals should be affirmed.

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